ZeeMed Healthcare Solutions

RSC CPT & Billing Configuration Review & Recommendations

Prepared for: **Ready Set Connect (RSC)**

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Status: Pre-Implementation / Billing Partnership Review

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Purpose
Initial review of CPT availability, service-to-CPT mappings, provider configuration, authorization considerations and recommended next steps for the RSC EMR and Practice Management platform.

Confidential Business Document

Prepared for discussion and implementation planning between RSC and ZeeMed.

1. Executive Summary

ZeeMed reviewed the CPT/service configuration questions raised by RSC and the provider feedback regarding missing occupational therapy and therapy-related codes. The immediate recommendation is to correct and complete the Stage configuration so providers can select applicable services and authorized CPTs without unnecessary support requests.

The longer-term recommendation is to keep the provider-facing service catalog simple while treating the CPT shown with a service as a default association. Final claim-level coding should remain subject to documentation, authorization, units, modifiers and payer requirements. This allows RSC to maintain a clean provider experience while ZeeMed supports the downstream billing and RCM process.

Our approach: establish the basics first, validate them in Stage, move the validated configuration to Live, and then develop additional recommendations as real provider and payer workflows generate useful data.

2. Scope of Review

• Missing CPT codes reported by RSC.

• Existing OT service-to-CPT mappings supplied by RSC.

• OT evaluation code availability.

• Provider-facing service and rate configuration.

• Authorization-to-CPT considerations.

• Fee schedule and reimbursement considerations.

• Recommended Stage-to-Live implementation sequence.

3. CPT Codes Requiring Review / Activation

CPT	Service / Description
97110	Therapeutic Exercise
97112	Neuromuscular Re-education
97166	OT Evaluation – Moderate Complexity
97167	OT Evaluation – High Complexity
97530	Therapeutic Activities
97535	Self-Care/Home Management Training
97537	Community/Work Reintegration
97750	Physical Performance Test
97755	Assistive Technology Assessment
97129	Therapeutic Interventions – Initial
97130	Therapeutic Interventions – Add-on

OT evaluation family: 97165 (Low Complexity), 97166 (Moderate Complexity), 97167 (High Complexity), and 97168 (Re-evaluation). RSC reported 97165 as already available; the remaining applicable levels should be available where supported by the provider's specialty and billing workflow.

4. Existing OT Service Mappings

RSC currently has several OT-facing services associated with 97530, including OT Treatment, OT Feeding, OT Consult, OT ESY and multiple OT Class services. ZeeMed recommends keeping the provider-facing service names because they are useful for service selection, but avoiding a design in which the service label alone locks every future claim to one CPT.

Provider-facing service	Default association	Billing-stage approach
OT Treatment	97530	Validate against documentation and authorization
OT Feeding	97530	Validate actual service before claim submission
OT Consult	97530	Review actual service and applicable payer rules
OT ESY	97530	Keep program/service label; validate claim CPT
OT Class services	97530	Review group/direct service circumstances before billing

5. CPT, Provider Rate & Reimbursement

CPT code ≠ universal reimbursement rate. A CPT identifies a service; actual reimbursement can vary by Medicare locality, Medicaid program/state, commercial payer, provider contract, fee schedule, authorization and applicable payer policies.

For example, an RSC provider may configure **OT Treatment → 97530 → $60**. The $60 should be understood as the provider's configured service rate, not as a universal U.S. reimbursement amount. ZeeMed can validate expected billing/reimbursement requirements at the payer and contract level during the billing process.

6. Recommended Provider-to-Billing Workflow

Provider Profile	Service Selection	Default CPT / Rate	Authorization
Clinical Documentation	Encounter / Charge Capture	Coding & Billing Validation	Claim Submission
ERA / Payment	Denial Management	A/R Follow-Up	Reporting

The objective is to keep RSC's provider workflow straightforward while allowing the downstream billing process to validate the actual claim requirements before submission.

7. Coding & Reimbursement Reference Framework

Reference	Role in ZeeMed's review
AMA CPT	CPT code set, descriptors and official CPT resources.
CMS Medicare	Physician Fee Schedule, therapy billing guidance and Medicare payment resources.
CMS NCCI / MUE	Correct-coding edits and related Medicare claim-editing guidance.
ICD-10-CM	Official diagnosis coding resources and guidelines.
CMS Medicaid NCCI	National Medicaid NCCI framework, supplemented by state requirements.
Commercial Payers	Payer-specific policies, authorization requirements and contracted fee schedules.

For production use, RSC should also confirm that CPT content displayed electronically is appropriately licensed for its EMR/business use. ZeeMed's billing operations can work from the applicable licensed/current data and payer-specific sources.

8. Immediate Stage-to-Live Recommendations

1. Add/enable missing CPTs applicable to RSC's supported specialties.

2. Review existing service-to-CPT mappings, particularly OT services.

3. Ensure applicable OT evaluation levels are available.

4. Confirm providers can select CPTs included in their authorizations.

5. Test provider service and rate configuration.

6. Validate the workflow in Stage before production deployment.

7. Monitor initial billing results and make controlled corrections where needed.

9. Longer-Term ZeeMed Recommendations

The immediate priority should be to establish a stable foundation for CPT, service and authorization configuration within the RSC EMR and Practice Management platform. As the platform moves into live billing and ZeeMed gains additional operational experience with providers, documentation patterns and payer workflows, we expect to identify further opportunities for improvement.

Those future recommendations may include additional billing workflow improvements, authorization controls, claim validation, reporting, denial/A/R processes and provider-facing enhancements. We recommend introducing these progressively based on actual operational requirements rather than overcomplicating the platform at the outset.

10. ZeeMed's Role in the Billing Partnership

RSC can continue focusing on its EMR, Practice Management platform and provider growth, while ZeeMed provides the specialized billing and RCM capabilities required downstream. This creates a clear division of responsibilities and allows the billing operation to scale alongside RSC's provider base.

RSC: EMR / Practice Management platform, provider experience and technology.
ZeeMed: Coding and billing expertise, claim operations, payer follow-up, denial management, A/R and revenue-cycle support, subject to the finalized partnership scope.

11. Conclusion

The current CPT request should be treated as the first step in establishing a reliable billing foundation for RSC providers. ZeeMed recommends completing the fundamental configuration first, validating it in Stage, and then expanding the billing workflow based on real-world requirements.

We view this as an ongoing collaboration. As the RSC EMR and Practice Management platform matures and the billing operation scales, ZeeMed can continue providing additional recommendations that improve provider experience, billing accuracy and revenue-cycle performance.

12. Scope & Use of This Document

This document represents ZeeMed's initial billing and RCM recommendations based on the CPT/service information provided for review. It is intended for implementation planning and discussion between RSC and ZeeMed. Final claim coding and reimbursement remain subject to the actual clinical documentation, applicable CPT/ICD-10-CM guidance, authorization, payer policy, provider contract and other applicable requirements.

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